

CITATION: Mason v. MacMarmon Foundation, 2011 ONSC 5823

COURT FILE NO.: 75749/05

DATE: 20111011

ONTARIO

SUPERIOR COURT OF JUSTICE

BETWEEN:

CHRISTOPHER MASON

Plaintiff

– and –

MacMARMON FOUNDATION o/a
BAYVIEW GLEN SCHOOL and MRS.
KURTZ

Defendants

Ronald P. Bohm, for the Plaintiff

Jennifer L. Hunter, for the Defendants

HEARD: September 29, 2011

**RULING ON MOTION FOR FURTHER DEFENCE MEDICALS AND
SUPPLEMENTAL EXAMINATION FOR DISCOVERY**

BOSWELL J.

Introduction:

[1] Christopher Mason was part of a school trip to a ski resort in Quebec in early February 2003. While boarding in the terrain park he fell and struck his helmetless head on a metal rail. His injuries are described in the Statement of Claim as including a concussion and brain injury, chronic pain and depression. The Defendants move, just weeks before the scheduled trial date, for an order compelling the Plaintiff to attend defence medical examinations with a physiatrist and a neuropsychologist and to re-attend for a supplemental examination for discovery. The Plaintiff opposes the relief sought.

Chronology:

[2] To understand the positions of the parties, it is necessary to put their current dispute into context, and to do so it is necessary to review a chronology of the medical examinations done to date.

[3] The physicians engaged in one form or another include the following:

- Dr. V. Kekosz – the Plaintiff’s treating physiatrist;
- Dr. H. Berry – the Plaintiff’s consulting neurologist;
- Dr. N. Doxey and Dr. G. Mitson – the Plaintiff’s consulting psychologists;
- Dr. M. Ford – the Defendants’ consulting orthopedic surgeon;
- Dr. S. Connell – the Defendants’ consulting psychiatrist;
- Dr. T. Smith – the Defendants’ consulting psychologist;
- Dr. H. Scher – the Plaintiff’s consulting neuropsychologist; and
- Dr. B. Stein, a treating psychiatrist of the Plaintiff (including prior to the accident).

[4] The action was commenced in May 2005. An Affidavit of Documents was delivered by the Plaintiff in early 2006, along with physician and hospital clinical notes and records. The Plaintiff was first examined for discovery on June 1, 2006. The examination continued on September 10, 2007.

[5] The first expert medical report was served by the Plaintiff in May 2007. It was a report of Dr. Berry dated January 17, 2007. The Plaintiff was referred to Dr. Berry by Mr. Bohm. The assessment occurred on September 21, 2006, some 3 ½ years post-accident. Dr. Berry took a history from Mr. Mason and, in addition, had a number of medical and other records available to him for review. A paediatric neurology report of Dr. Betty Koo dated February 13, 2003 indicated that Mr. Mason had lost consciousness for about a minute after the accident, then had a period of about 17 hours when he was “in and out of awareness”. An MRI brain scan taken May 20, 2003 did not reveal any brain abnormalities.

[6] Dr. Berry found, based on his review, that the Plaintiff was involved in a snowboarding accident on February 2, 2003, as a result of which he suffered a concussion with an interval of amnesia lasting up to several days in duration. He experienced headaches, nausea, neck and back pain. He returned to school after two weeks but remained lethargic and dazed, with considerable pain. Symptoms of depression along with headaches, intellectual difficulties and musculoskeletal pain and troubled emotions became prominent after the accident. He encountered difficulty with his schooling and eventually took a year off and travelled.

[7] Pre-accident records revealed that he had experienced depression in the months prior to the accident and psychotherapy had been recommended. A North York General Hospital

psychiatric report of January 21, 2003 (authored by Dr. Stein) revealed a diagnosis of dysthemia, and an avoidant and passive-aggressive personality.

[8] Dr. Berry suggested two possible diagnoses: (1) a more severe form of concussion with permanent brain injury; or (2) an emotional reaction to the accident and a mild concussion without brain damage in a vulnerable patient with an ongoing pre-accident depression. He favoured the diagnosis of a concussion with a degree of permanent brain damage. He opined that the Plaintiff's pre-accident potential for higher education, more challenging intellectual tasks, new learning and his range of career choice have been diminished by the effects of permanent brain injury, but it was suggested that he should be able to function at a lower level of achievement and challenge during his future years.

[9] On September 21, 2007, the Plaintiff served a psycho-vocational assessment report of Dr. Doxey dated August 27, 2007. Dr. Doxey's assessment was conducted with the assistance of Dr. Mitson. They concluded that the accident has had significant impacts on the Plaintiff's physical, cognitive and psychological functioning. They found that the Plaintiff met the DSM-IV criteria for "Pain Disorder Associated with a General Medical Condition – Chronic" as well as "Cognitive Disorder Not Otherwise Specified." In the course of their assessment, they noted that the Plaintiff presented as a severely brain damaged individual who often struggled to understand questions, had difficulty expressing himself and displayed significant memory problems. He appeared to have, at a minimum, significant verbal memory deficits. They opined that the Plaintiff's symptoms arose from a probable brain injury sustained in the snowboarding accident, but they also recommended that a neuropsychological assessment be conducted, which may reveal areas of cognitive dysfunction.

[10] On December 14, 2007, with the reports of Dr. Berry and Dr. Doxey in hand, the Defendant sought two defence medical examinations, one by Dr. Ford and the other by Dr. Connell. The Plaintiff agreed to attend both examinations.

[11] On January 28, 2008 Dr. Ford produced a report in which he said noted that, on examination, the Plaintiff had a full range of motion in his cervical spine and shoulders. He had a normal range of motion in his lumbar spine. A review of the Plaintiff's medical history revealed a prior history of back pain. An MRI done in May 2003 showed no abnormalities save for a small cyst at L1-2, which was considered clinically insignificant. It is not clear what medical records Dr. Ford had and/or reviewed in relation to the Plaintiff. It is apparent, however, from comments made in his report, that he did have and review clinical notes and records of Dr. Kekosz, the Plaintiff's treating physiatrist. Dr. Ford found no objective evidence of physical impairment and opined that the Plaintiff had suffered soft tissue injuries only. There was, accordingly, no need for formal medical therapy.

[12] On November 20, 2008, Dr. Connell produced a report in which he diagnosed "Dissociative Disorder, not Otherwise Specified; Pre-existing Dysthemia; possibly Bipolar". Dr. Connell had an opportunity to meet with and assess the Plaintiff. He conducted, amongst other tests, a mini mental status test in which the Plaintiff scored 27 out of 30. He was also able to review a transcript of the Plaintiff's examination for discovery, as well as the Plaintiff's expert

report brief and a psychological brief. He had Dr. Stein's notes, the emergency records, Dr. Kekosz's notes, and the reports of Dr. Berry and Dr. Doxey discussed above.

[13] Dr. Connell opined that the accident had not contributed to the Plaintiff's psychiatric course, apart from adding to his burdens. His psychological disorder is, in Dr. Connell's view, unrelated to the accident. Instead, it is Mr. Mason's psychiatric disorder – diagnosed by Dr. Stein in January 2003 – that has resulted in his chronic psychological and cognitive difficulties. In Dr. Connell's opinion, Dr. Berry and Dr. Doxey do not give sufficient consideration, in their respective reports, to the predominance in the Plaintiff's history of his dissociative symptoms and profound disturbance of self-experience and memory.

[14] Dr. Doxey prepared a short report in June 2009 in which he noted that Dr. Stein had seen the Plaintiff at age 17. He considered Dr. Connell's opinion to be extreme and unrealistic and suggested that "Dr. Connell's formulation did not conform to a number of the objective facts of the Plaintiff's life prior to the accident."

[15] On September 27, 2010, Dr. Kekosz delivered a report in which she stated that the Plaintiff had suffered a head, neck and lower back injury as a result of the snowboarding accident. An MRI scan of his neck did not reveal any disc herniation, but there was a question of ligamentous instability at C2-3 on his initial flexion and extension x-rays of the cervical spine. He also showed evidence of an L4-5 bulging disc and EMG evidence of an L5 nerve root irritation in the right lower extremity. He also sustained a labral tear in the right shoulder joint for which surgical intervention was advised. In addition to physical impairments, she said the Plaintiff suffered the sequelae of a probable mild traumatic brain injury and post-concussion headaches. She pointed to, as evidence to support that diagnosis, problems with short and long term memory and to a psycho-vocational assessment that evaluated his cognitive difficulties. She was referring, of course, to Dr. Doxey's assessment. Dr. Doxey recognized, however, the shortcomings of his assessment and had recommended a full neuropsychological assessment to determine more reliably whether there are areas of cognitive dysfunction.

[16] On October 13, 2010, Dr. Ford produced a supplemental report in which he commented on the labral tear noted by Dr. Kekosz. He said he had difficulty understanding how Mr. Mason hurt his shoulder. This was, he noted, a snowboarding accident where the Plaintiff hit his head. He also noted that he sees, in his practice, individuals with far more severe head traumas, who generally display a low incidence of prolonged neck pain. He re-affirmed his earlier findings.

[17] Subsequently, on October 25, 2010, the report of Dr. Smith was produced. He is a psychologist retained by the Defendant to conduct a paper review of the medical reports and records to date and to provide an opinion. He reviewed the medical brief, including hospital records, the expert reports of both sides, a damages brief and discovery transcripts. He opined that Dr. Berry's opinion of permanent brain injury is unfounded. He also noted that it was incredible that a neuropsychological evaluation had not been conducted.

[18] Dr. Smith observed that the Plaintiff's academic and vocational path clearly changed post-accident, but noted that the etiology of those changes remained confused. In his view, the neurocognitive sequelae of the head injury should have cleared within a matter of weeks or months. Instead, the Plaintiff's reported deficits got worse, to levels suggesting either deliberate dissimulation or else medication effects or psychiatric illness.

[19] It appears that Dr. Doxey, Dr. Connell and Dr. Smith all agreed that a neuropsychological assessment should be conducted to assist in concluding whether the Plaintiff suffers any cognitive deficits that may reasonably be concluded to be sequelae of the accident.

[20] The action was set down for trial in April 2008. The trial date was fixed for the Fall 2009 trial sittings. For reasons not entirely clear to me, the action was mistakenly struck from the trial list and subsequently re-listed for trial in the Fall 2010 sittings. The trial did not proceed in the Fall 2010 sittings due to defence counsel's scheduling conflict. It was ultimately put over to the Fall 2011 sittings.

[21] In the meantime, the Defendant requested, in February 2011, that the Plaintiff attend two further defence medical examinations. Specifically, the Defendants concluded, following receipt of Dr. Smith's report, that a neuropsychological examination was necessary. Defence counsel arranged for the Plaintiff to attend a consultation with Dr. Lorraine McFadden, a neuropsychologist. In addition, they wanted the Plaintiff to attend a further defence medical examination with a psychiatrist retained by the defence, namely Dr. Rajka Soric. The Plaintiff refused to attend the assessments on the basis that the Defendants had already elected to have the Plaintiff examined by an orthopedic surgeon and a psychiatrist and were not entitled to any further defence medicals.

[22] The defence then produced a further report of Dr. Connell dated April 8, 2011. Dr. Connell reviewed the additional medical evidence accumulated since his earlier report. He reiterated that Mr. Mason's psychological difficulties were evolving prior to the snowboarding accident. His impairments and earning capacity are the result of his longstanding psychological mechanisms and choices and not from any psychiatric injury sustained in the accident. He referenced Dr. Smith's observation that "rather remarkably, a comprehensive neuropsychological assessment, which would have been the only reliable way of addressing the functional limitations of any specific deficit and delineating their functional limitations has never been done."

[23] Subsequently, the Plaintiff delivered to the Defendants, on July 21, 2011, a report from Dr. Scher, who conducted a neuropsychological examination of the Plaintiff. Dr. Scher's report really ignited the present dispute between the parties. Dr. Scher said that his findings strongly reinforce the impression that factors other than traumatic brain injury are causing serious impairment in cognitive functioning at this time. He added that the neuropsychological test results could not be used to infer the presence of any traumatic brain injury sequelae that might be present. He went on to say, however, in what might arguably be inconsistent fashion, that his diagnosis was "Mild Traumatic Brain Injury with an uncertain degree of full neurocognitive recovery". To add a further layer of complication, he opined that regardless of whatever brain injury was sustained (or not sustained), the Plaintiff is unable to demonstrate normal levels of

cognitive efficiency and productivity as a result of a serious chronic pain condition that is causally related to the subject accident.

[24] This motion was argued on September 29, 2011. Remarkably, on September 30, 2011, the Defendants' counsel received from the Plaintiff's counsel, two further medical reports, which led to further, written submissions from counsel. One of the additional reports was from Dr. Berry, who gave his updated views after having had an opportunity to review Dr. Scher's report. Dr. Berry indicated that, like other tests, neuropsychological assessments are of limited usefulness by themselves and must be interpreted in conjunction with clinical and other information. In this case, the diagnosis of brain damage arises out of the event of the closed head injury with concussion, an observed interval of confusion, the prolonged duration of the post-traumatic amnesia and the subsequent symptoms and difficulties. He confirmed his earlier diagnosis: that Mr. Mason suffered a permanent brain injury with significant deficits which are contributed to by the post-traumatic emotions of depression and anxiety and also by ongoing back and neck pain as a result of the accident.

[25] The other report was from Dr. Stein, the psychiatrist who treated the Plaintiff before the accident giving rise to this lawsuit. Dr. Stein provided an indication of his assessment of the Plaintiff pre-accident and in addition, provided an opinion about the views expressed by the Defendants' expert, Dr. Connell. Dr. Stein said he would not agree with Dr. Connell's opinion that the Plaintiff's childhood was so traumatic as to cause him to have little recall of his childhood or teenage years. He suggested that there is insufficient evidence from which one might reasonably conclude that the Plaintiff would essentially inevitably experience a psychological deterioration of the sort witnessed here. He said little about the value of a neuropsychological assessment, other than to say that he has requested it in certain patients and found it to be helpful in eliciting areas of neurological dysfunction.

Positions of the Parties:

[26] I heard extensive argument from counsel to both sides. Both counsel made well-prepared, thorough and compelling arguments about the need and/or entitlement to the further defence medicals.

[27] The Defendants assert that the delivery of Dr. Scher's report has significantly changed the landscape of this litigation. In the face of a refusal to permit a defence-initiated neuropsychological examination, the Plaintiff delivered Dr. Scher's report, just three months prior to the scheduled trial date. Prior to Dr. Scher's report, the defence believed that the Plaintiff had not marshaled sufficient evidence to meet his evidentiary burden to establish that his purported psychological deficits were the sequelae of a brain injury suffered in a snowboarding accident. The Defendants had confidence that it would be enough for them to point to the Plaintiff's failure to obtain a neuropsychological assessment, in the face of a number of experts who commented on its necessity, in order to undermine the Plaintiff's position on causation.

[28] Dr. Scher's report represents, in the view of the Defendants, a significant shift. First, they can no longer argue that the Plaintiff has failed to obtain a neuropsychological report.

Second, the author of the report opines that the Plaintiff has indeed suffered a mild traumatic brain injury. Third, Dr. Scher goes on to opine, perhaps even more significantly, that the Plaintiff's cognitive deficits are the result, at least in part, of a serious chronic pain condition that is causally related to the snowboarding accident.

[29] The Defendants now believe they are seriously on the back foot, without a neuropsychologist to answer Dr. Scher and without a physiatrist's report to comment on the serious chronic pain condition referred to by Dr. Scher. Moreover, they believe that observations made by Dr. Scher during the course of his assessment reflect a serious deterioration in the Plaintiff's condition, such that a further examination for discovery is necessitated. Trial fairness dictates, in their view, that they be entitled to a further discovery and that the Plaintiff be compelled to attend the two further defence medical assessments they have arranged.

[30] The Plaintiff's position may be stated more briefly. The Plaintiff asserts that the Defendants have had all of the assessments they are entitled to. They chose the timing of their request for assessments and now must live with it. A defendant has no right, the Plaintiff argues, to obtain matching reports. The Defendants selected their experts for tactical reasons and must now live with their results.

[31] The Plaintiff argues that the Defendants' experts have fully opined on the Plaintiff's injuries and no further defence medicals are warranted, nor necessary to ensure trial fairness.

Issues:

[32] The following issues were raised for determination:

- (i) A threshold issue regarding whether the Defendants should be granted leave to argue the motion in view of the fact that the matter has been set down for trial, with their consent;
- (ii) Whether the Plaintiff should be compelled to attend any further defence medical examinations; and,
- (iii) Whether the Defendants are entitled to a further cross-examination of the Plaintiff.

[33] I will review these three issues in turn.

Analysis:

(i) *Should leave be granted?*

[34] Rule 48.04 provides that any party who has consented to a matter being placed on the trial list is precluded from initiating or continuing any form of motion or discovery without leave of the court. As I observed in *Kernohan v. York (Municipality)*, 2009 CanLII 9422, granting leave is a discretionary remedy, to be exercised having regard to the general principle set out in rule 1.04, that the *Rules of Civil Procedure* "shall be construed liberally to secure the just, most expeditious and least expensive determination of every civil proceeding on its merits."

[35] Where proposed motions involve matters affecting substantive rights, or trial fairness, leave is frequently granted. In the context of a personal injury action, trial fairness issues may be engaged where there has been a change in a Plaintiff's condition after defence medical examinations have been conducted. Alternatively, the delivery of further experts' reports by the Plaintiff after the trial record is passed may create an uneven playing field, justifying at least a consideration of a motion for leave to conduct further examinations. See for example, *MacRae v. Dreuniok*, 2007 CanLII 36080 (S.C.J.); *Tanner v. Clark*, [1999] O.J. No. 581 (S.C.J.); *Kinch v. Walden*, [2011] O.J. No. 2186; *Aljoe v. Co-operators General Insurance Co.*, 2005 CarswellOnt 2925; *Ortiz v. Sharma*, [2007] O.J. No. 2984. To refuse leave in circumstances where trial fairness is at stake, would permit form to triumph over substance.

[36] In this instance, the extent and cause of the Plaintiff's psychological deficiencies are very much live issues. Experts for both parties indicated that a neuropsychological assessment was required to address the nature and extent of any cognitive defects of the Plaintiff. The Plaintiff appeared not to be prepared to obtain such an assessment and, in February 2011, refused to participate in an assessment organized by the Defendant. Subsequently, and just within the notice periods dictated by the Rules, the Plaintiff served an assessment report of a neuropsychologist. The report closes a significant gap in the Plaintiff's case and in addition includes an opinion that even if the Plaintiff's psychological defects are not the sequelae of a traumatic brain injury, they are inextricably intertwined with the chronic pain suffered by the Plaintiff since the accident.

[37] Whether the Defendants are, in the circumstances, entitled to a further medical examination in light of these developments is a matter that, in my view, affects the issue of trial fairness and, in the result, leave is granted to the Defendants to proceed with their motion.

[38] The Plaintiff cited the case of *Hill v. Ortho Pharmaceutical (Canada) Ltd.*, [1992] O.J. No. 1740 (S.C.J.) arguing that a demonstrated substantial and unexpected change is a precondition to the granting of leave. Rule 48 does not require that such a change be demonstrated. Leave must be granted, in accordance with the primary objective of the Rules, to ensure the just, most expeditious and least expensive resolution of proceedings. Issues of justice and fairness must be foremost in the court's mind when exercising its discretion. In my view, a substantial and/or unexpected change is certainly one factor that the court will consider, but not the only factor.

[39] *Hill* was a case where the plaintiffs sought to vacate a Certificate of Readiness, so that they could conduct further discoveries. In my view, further discoveries ought not to be permitted in the absence of a significant and unexpected change in circumstances, but I do not agree that a significant change is a pre-requisite to the granting of leave to hear any motion after a Trial Record is passed.

[40] In any event, even if a significant change is required, I would find that it exists here. The delivery of Dr. Scher's report was entirely unexpected in all the circumstances and did, in my view, change the landscape considerably in terms of the evidence relating to the Plaintiff's psychological injuries.

[41] Leave to proceed with the motion is granted.

(ii) *Should the Plaintiff be compelled to attend further defence medical examinations?*

[42] Section 105(2) of the *Courts of Justice Act*, R.S.O. 1990 c. C.43 provides that where the physical or mental condition of a party to a proceeding is in question, the court may order the party to undergo a physical or mental examination by one or more health practitioners. Rule 33.02(2) of the *Rules of Civil Procedure* provides that the court may, on motion, order a second or further medical examination(s) on such terms as are just. The upshot of these provisions is that an order compelling a plaintiff to attend a second or subsequent defence medical examination is a discretionary matter.

[43] The exercise of the court's discretion involves a consideration of a number of factors, including, but not limited to the following:

- (i) The need for the examination, keeping in mind that the onus is on a defendant to satisfy the court as to its necessity: *Jones v. Spencer*, [2005] O.J. No. 1539 (S.C.J.); *Francescutto v. Bojsic*, [1999] O.J. No. 602 (S.C.J.); *Moore v. Royal Insurance Co. of Canada*, [2006] O.J. No. 166 (S.C.J.);
- (ii) The relative prejudice to the parties in allowing or not allowing the further examination(s): *Chiarelli v. Weins*, [2000] O.J. No. 296 (C.A.); *MacRae v. Dreuniok*, as above;
- (iii) The number of defence medical reports already obtained, keeping in mind that the defence are not automatically entitled to "match" a plaintiff's experts: *Burden v. Shoebottom*, 2004 CanLII 24395 (S.C.J.);
- (iv) The delay and increased costs that may be occasioned by the order for a further medical assessment and whether permitting further examination(s) would be consistent with the principle of keeping costs and the length of the proceeding proportionate to the amount or issues at stake;
- (v) Whether fairness and justice require that a further defence medical examination be sanctioned. In other words, whether trial fairness would be prejudiced in the absence of the further examination(s): *Kernohan*, as above; *Ortiz v. Sharma*, [2007] O.J. No. 2984 (S.C.J.).

[44] In this instance, there are two distinct examinations sought. I will deal with them in turn, beginning with the request for a further examination by a physiatrist.

[45] The Plaintiff alleges that he suffered injuries to, *inter alia*, his neck, back and shoulders. He complains of chronic pain. The Plaintiff has produced notes and records of his consulting physiatrist, Dr. Kekosz, as well as formal reports prepared by her. The Defendants elected early on to take the approach that the Plaintiff suffered only soft tissue injuries and has no objective musculoskeletal impairments. It is not uncommon for a defendant to respond to an allegation of chronic pain with the evidence of an orthopedic surgeon who testifies about the absence of

objectively identifiable medical impairments. The assertion is typically that the Plaintiff is either malingering or otherwise has the same types of aches and pains that many people suffer from on a daily basis.

[46] There are no significant changes in the Plaintiff's physical condition, or in the medical evidence surrounding his physical condition, since the Defendants had him assessed by Dr. Ford. It was the Defendants' choice, for tactical reasons, to have the Plaintiff assessed initially by an orthopedic surgeon and not a physiatrist. In my view, they are bound to that choice, for better or worse.

[47] I see no reason why, in the interests of fairness, a further defence medical examination is required in terms of the Plaintiff's physical injuries. I see the current request as a mere attempt to match up experts. The lack of a defence physiatrist's report is a function of the Defendants' tactical choices and will not render the trial less than fundamentally fair.

[48] The request for a further defence medical examination with a physiatrist is denied.

[49] The situation with respect to the neuropsychologist is different, in my view. I do grant the Defendants' motion that the Plaintiff be compelled to attend for a defence neuropsychological examination, for the following reasons:

- (i) The cause of the Plaintiff's psychological impairments is very much a live issue. A number of experts agree that a neuropsychological assessment is advisable, if not necessary, to determine whether the Plaintiff has cognitive deficits and what the nature and extent of those deficits is;
- (ii) At the time the Defendants selected their defence medical examiners (Dr. Ford and Dr. Connell) they had Dr. Doxey's report of August 27, 2007 in which he recommended a neuropsychological report. Arguably they would have been alive to the need for such a report at that time. That said, the Plaintiff had not, to that point, obtained such a report and in fact did not do so for almost four more years. Moreover, there was evidence in the form of the notes and records of Dr. Stein, that indicated the Plaintiff had a pre-existing underlying psychiatric condition. It made sense that they asked the Plaintiff to meet with their consulting psychiatrist;
- (iii) The Plaintiff denied the Defendants' request for a neuropsychological assessment in February 2011. That date was some 8 years post-accident. It was reasonable for the Defendants to believe that the Plaintiff did not intend to obtain a neuropsychological report in view of the fact that he had not done so in 8 years, coupled with his refusal to attend on such an examination arranged by the Defendants. I accept that the Defendants were taken completely by surprise when they received Dr. Scher's report in July 2011;
- (iv) Dr. Scher's report leaves the etiology of the Plaintiff's psychological impairments in a state of confusion. Although he finds that the tests he utilized can not be used to infer the presence of any traumatic brain injury sequelae, he still opined that the Plaintiff suffered from a mild traumatic brain injury;

- (v) Dr. Scher also added a layer of complexity by opining that the Plaintiff's diminished cognitive functioning is the result of a serious chronic pain condition causally connected to the snowboarding accident. The Plaintiff's neurologist, Dr. Berry, had previously opined that the Plaintiff's emotional condition (depression, irritability and frustration) was attributable to the ongoing pain and limitations caused by the accident and he added that a significant component of the emotional condition was likely attributable to a permanent brain injury. Whether Dr. Berry is ultimately qualified to opine on the cause of the Plaintiff's depressed condition is debatable. But what he did *not* say was that the Plaintiff suffered cognitive deficits (such as memory problems) as a result of chronic pain. Dr. Scher added that level and it is an important one; and,
- (vi) The Defendant has arranged for a neuropsychological examination on October 13, 2011 (in other words, within a matter of days) and it is anticipated that a report will be generated in time for trial, such that the trial will not inevitably be delayed.

[50] I am satisfied, on balance, that the prejudice to the Defendants in not allowing the examination outweighs any prejudice to the Plaintiff in allowing the examination to proceed. In the interests of trial fairness, the Plaintiff is directed to attend a neuropsychological examination with Dr. Lorraine McFadden on October 13, 2011.

(iii) *Should the Plaintiff be compelled to attend a further examination for discovery?*

[51] The short answer to this question is no. In *Suchan v. Casella* (2006), 81 O.R. (3d) 61, Master Dash held that exceptional circumstances, such as a substantial deterioration in a plaintiff's condition, are necessary before the court will allow a further examination of the plaintiff for the purpose of updating the plaintiff's condition. I am not satisfied, on the record before the court, that there has been a substantial deterioration in the Plaintiff's condition, nor are there any other exceptional circumstances present that would justify a further examination for discovery.

[52] In this instance, the evidence suggests that a continued examination would be of minimal assistance in any event. The Plaintiff suffers, allegedly, from a traumatic brain injury. Whatever the cause of his psychological impairments, it is apparent that he has memory deficits and has a difficult time answering questions. The value of a further examination is questionable. In the absence of a very compelling reason, a further oral examination of him is not justified. I am not satisfied that such a compelling reason exists here.

[53] I am also mindful that the Defendants will have the benefit of a very current neuropsychological assessment. Any concerns about up-to-date information relating to the Plaintiff's psychological condition will be alleviated to a great degree by the current assessment.

Conclusion:

[54] In the result, the Defendants' request for a further examination for discovery is dismissed. The Defendants' request for a defence examination with a psychiatrist is dismissed. The request

for a defence neuropsychological assessment is granted and the Plaintiff is directed to attend before Dr. Lorraine McFadden on October 13, 2011 for such an assessment.

[55] The parties may make written submissions on the issue of costs, on a 14 day turnaround, beginning with the Defendants, whose submissions are to be delivered by October 24, 2011. Submissions are not to exceed 2 pages in length and are to be submitted through the judicial secretaries at Newmarket.

Boswell J.

Released: October 11, 2011